

PERFORMANCE WORK STATEMENT

for

Military Treatment Facilities
Third Party Collections Program



Revenue Cycle Management Branch
Cost Accounting Division, Financial Operations J-8
Defense Health Agency
Department of War

TABLE OF CONTENTS

1. DESCRIPTION OF SERVICES	5
1.1 Background	5
1.2 Scope	5
1.3 Implementation	5
2. GENERAL INFORMATION	5
2.1 Contractor Identification	5
2.2 Contractor Training	6
2.3 Contractor and Government Communication	6
2.4 Place of Performance	7
2.5 Travel Requirements	7
2.6 Mission and Emergency Essential	7
2.7 Duty Hours	7
2.8 Federal Holidays	7
2.9 Conduct of Contractor Personnel	7
2.10 Health Requirements	8
2.11 Access to GBS	9
3. SECURITY	9
3.1 Security Requirements	9
4. CONTRACT REQUIREMENTS	11
4.1 Third Party Collections Services	11
4.2 Cease of Collection Activities	12
4.3 Claim Generation and Submission	12
4.4 Insurance Verification	12
4.5 Preauthorization and Advance Notification	13
4.6 Encounter Coding/Data Review	13
4.7 Payment Adjudication Monitoring	13
4.8 Balance Billing	14
4.9 Follow-Up/Denials Management	14
4.10 Unpaid Claims	14
4.11 Payment Posting and Refund Management	14
5. QUALITY CONTROL PLAN (QCP)	15
5.1 QCP	16
6. POST AWARD AND INITIAL CONTRACT PERFORMANCE	16
6.1. Post Award	16
6.2 Business Process	17
6.3 Human Resource Management Plan	17
6.4 Medical Expense and Performance Reporting System (MEPRS)	17
6.5 Invoices	17

7. TRANSITION/CONTINGENCY PLANS	17
7.1 Phases-in Plan	17
7.2 Phase-out Plan	17
7.3 Contingency Plan	18
8. CONTRACTOR QUALIFICATIONS	18
8.1 Experience and Requirements	18
8.2 Personnel Qualifications	18
8.3 Certification Statement	19
8.4 Government Furnished Training	19
8.5 Continuing Training	19
8.6 Identification of Contractor Employees	19
9. CONTINUITY OF SERVICES	20
9.1 Continuity of Services	20
9.2 Subcontracting	20
10. ADDITIONAL ADMINISTRATIVE CONSIDERATIONS	20
10.1 Contract Management	20
10.2 Inspection and Acceptance	20
11. PII AND PHI	20
11.1 Safeguards	20
11.2 HIPAA	21
11.3 Breach Response	21
11.4 Business Associate Agreement	21
12. PERFORMANCE OBJECTIVES	21
12.1 Services Summary (SS)	21
12.2 Monthly Status Reports	21
12.3 Services Summary Table	22
13. GOVERNMENT FURNISHED PROPERTY AND SERVICES	23
13.1 General	23
14. DELIVERABLES	24
14.1 Aging of Accounts Receivable	24
14.2 DD2570 Report	24
14.3 Monthly Invoice	24
15. APPENDIX A ACRONYMS	25
16. APPENDIX B BUSINESS ASSOCIATE AGREEMENT	27
17. APPENDIX C DHA NON-DISCLOSURE AGREEMENT	36
18. APPENDIX D APPLICABLE PUBLICATIONS AND FORMS	37

19. APPENDIX E HEALTH INSURANCE PORTABILITY AND ACCOUNTABILITY ACT (HIPAA)

20. APPENDIX F CONTRACTOR FULL TIME EQUIVALENT REPORTING ADDENDUM

21. ATTACHMENT 1– LIST OF MTFs

DRAFT

THIRD PARTY COLLECTIONS PROGRAM

1. DESCRIPTION OF SERVICES

- 1.1. Background.** In accordance with U.S. Code 1095 and 1079b, the Code of Federal Regulations, and Department of War (DoW) directives, the Government is authorized to recover the cost of providing health care services to covered DoW beneficiaries from third party payers under the Third-Party Collections Program (TPCP). Under this program, the Government bills for medical services provided in DoW/Defense Health Agency (DHA) military treatment facilities (MTFs) and professional services rendered by MTF providers at civilian facilities. The Government also bills third party payers for care provided to non-beneficiaries who are authorized care in the facility, foreign nationals, and civilian emergency patients who carry Other Health Insurance (OHI). All patients, excluding active duty, are required to provide information regarding OHI coverage annually or upon a change in coverage status.
- 1.2. Scope.** The purpose of this contract is to centralize OHI billing and collection activities to standardize and optimize revenue within the enterprise using a government provided medical billing program (herein referred to as the Government Billing Solution or “GBS”). Billing and collection activities shall include: identification and verification of OHI provided on the paper and/or electronic DD Form 2569, updating OHI information into the GBS, direct billing of third party payers, ongoing follow-up actions for unpaid claims every 30 days to include denials management processes, posting payments, conducting valid write-offs and referral of delinquent claims by the 120th day delinquency. This contract will not include first party billing to beneficiaries, interagency billing, billing for Elective Cosmetic Surgery procedures, copying charges, or incidental charges (family member rate) for inpatient stays. The contractor shall be required to conduct balance billing for non-beneficiaries only. The successful Offeror shall:
- 1.2.1.** Adhere to all applicable U.S. Codes, the Code of Federal Regulations, DoW Guidance, Centers for Medicare and Medicaid Services guidance, and DHA Policy regarding billing and collection activities (those in effect at the time of contract award and any changes made throughout the contract period of performance).
- 1.2.2.** Adhere to all DoW and DHA published guidance and regulations regarding financial reporting.
- 1.3. Implementation.** This contract shall include all MTFs identified in Attachment 1 – List of Military Treatment Facilities (MTFs).

2. GENERAL INFORMATION

- 2.1. Contractor Identification.** Within 10 calendar days following notification of contract award, the Contractor shall provide, in writing to the Contracting Officer (CO) and the Contracting Officer’s Representative (COR), the name of the Contractor Representative responsible for coordination and implementation of contract services. Additionally, the representative shall be available telephonically to the CO/COR during normal duty hours to discuss any problems that may arise concerning contractual matters relating to daily operation of the contract.
- 2.1.1.** The Contractor shall notify the CO/COR at least five calendar days in advance of any change or replacement actions concerning the individual selected to be the Contractor

Representative after the contract period of performance begins. Such notification shall be in writing and shall state the name and telephone number of the new Contractor Representative.

- 2.1.2.** The Contractor Representative shall advise the CO/COR 30 calendar days in advance of any foreseeable event that could affect performance of these contract requirements. Personnel changes, to include scheduled and unscheduled absences of contract personnel, shall be reported to the COR as soon as possible. The Contractor Representative shall also provide a written plan for continued support until the departing Contractor employee is replaced.

2.2. Contractor Training. The contractor is required to complete any Government required Contractor training listed below:

- 2.2.1. Basic Life Support Certification.** At a minimum, any Contractor personnel working within an MTF must maintain either current certification in the American Heart Association Basic Life Support (BLS) or the American Red Cross Heart Saver Course. Initial training must be obtained in the community at no cost to the Government, so the Contractor employees arrive in the position fully qualified. The Government may offer refresher training for contract workers on a space available basis. If training is received within the facility, the time will not be billable to the Government. The Government will not pay for recertification training obtained outside the MTF.

- 2.2.2. Privacy Act, Health Insurance Portability and Accountability Act (HIPAA) and Cyber Awareness Training.** The Contractor shall ensure that all staff, including sub-contractors and consultants, comply with the training requirements of the Privacy Act of 1974 (5 U.S.C. 552a) and Health Insurance Portability and Accountability Act of 1996 (Pub. L. 104-191). The Contractor shall ensure annual Privacy Act and HIPAA training is completed by all staff assigned to this contract, including sub-contractors and consultants. All required Privacy Act and HIPAA training will be conducted online through Joint Knowledge Online (JKO) at <https://jkodirect.jten.mil> or the current Military Health System/DHA learning platform in place to deliver training to meet the above requirements. The Contractor shall ensure all employees and sub-contractors supply a certificate of Privacy Act, HIPAA, and Cyber Awareness training to the COR within 30 days of being assigned to the contract and on an annual basis.

- 2.2.3. GBS Training.** Specific training required for any access to the GBS.

- 2.2.4. Records Management.** When creating and maintaining official Government records, the Contractor shall comply with all federal requirements established by 44 USC, 41 USC, 36 CFR, DoD Administrative Instruction No. 15 (DoD AI-15), "Records Management, Administrative Procedures and Records Disposition Schedules," and Chapter 2 of the TRICARE Operations Manual.

2.3. Contractor/Government Communication. The Contractor shall designate a focal point to be the single point of contact for all Contractor and Government

correspondence. The focal point shall provide clear and consistent written and verbal response to the Government within **12 business hours** of Government initiated communication (e.g., return phone calls, emails or other communication). The Contractor shall communicate via Government email addresses and not company email address.

- 2.4. Place of Performance.** The Contractor shall determine the appropriate staffing and placement of contractor staff necessary to carry out the services under this contract in coordination with the COR.
- 2.5. Travel Requirements.** Travel may be required during the performance period of this contract at the request of the Government and within the Continental United States (CONUS) and Outside Continental United States (OCONUS).
- 2.6. Mission/Emergency Essential.** None of the services listed in this PWS are mission/emergency essential.
- 2.7. Duty Hours.** Normal duty hours are 7:30 am to 4:30 pm, Monday through Friday (excluding Federal Holidays).
- 2.8. Federal Holidays.** Federal offices are closed on federal holidays.
- 2.9. Conduct of Contractor Personnel.** The CO may require the Contractor to remove from the job site Contractor personnel working under this contract. Removal from the job site or dismissal from the premises shall not relieve the Contractor of the contract requirements.
 - 2.9.1.** The Contractor shall not employ individuals who are potential threats to health, safety, security, general well-being, or the operational mission of the DoW and its population. This shall include individuals who are a potential threat because of prior felony convictions (reference Automated Information Systems (AIS) III Security Requirements, section 3.1 below). The Government reserves the right to refuse access to Government facilities and/or operations under this contract to any Contractor employee, or prospective employee, who is identified as a potential threat to the health, safety, security, general well-being, or operational mission of the DHA and its population.
 - 2.9.2.** The Contractor shall ensure all Government-issued identification badges, keys, and equipment (as appropriate) are returned to the COR or MTF Security Manager when an individual leaves employment under this contract and/or upon expiration of this contract.
 - 2.9.3.** English Language Requirement: Contractor employees shall read, understand, speak, and write English fluently.
 - 2.9.4.** Contractor personnel shall be required to observe Government facility parking, safety and traffic regulations that apply to all facility employees.

2.9.5. Alcoholic beverages on the job are prohibited.

2.9.6. There shall be no loud, profane or abusive language used on the job.

2.9.7. Contractor personnel shall present a neat well-groomed appearance. Neat, clean, casual business attire clothing shall be worn.

2.10. Health Requirements. All Contractor employees shall follow the methods for controlling and preventing disease as described in the American Public Health Association publication, *Control of Communicable Diseases Manual*, and the Centers for Disease Control and Prevention (CDC) publication, *Morbidity and Mortality Weekly Report* (MMWR), and its supplements. Where applicable, the most recent guidelines from these publications are utilized as the standard.

2.10.1. Prior to start of work, Contractor employees working in MTFs shall provide proof of immunization for the following diseases according to CDC guidelines: Hepatitis B, measles, mumps, rubella, varicella, and influenza, and proof of a negative tuberculosis skin test completed within the past 12 months (if positive, proof of negative chest X-ray within the past 12 months is required). After work starts, the Government will provide post bloodborne exposure protocols according to applicable regulations.

2.10.2. In those areas where there is a higher risk of transmission of tuberculosis, Contractor employees may be tested as frequently as directed by the MTF policy. This test will be provided by the MTF at no cost to the Contractor.

2.10.3. Immunization information will be tracked in DoW computer systems for all Contractor employees providing services within the MTF under this contract.

2.10.4. No medical tests, procedures, or immunizations required by the contract will be performed by the MTF (with the exception of tuberculosis testing after start of work and in the event of exposure). Expenses for all required tests and/or procedures shall be borne by the Contractor, not the Government.

2.10.5. Contractor employees working in an MTF are strongly encouraged to be immunized annually with the influenza vaccine. This vaccine will not be provided by the Government to Contractor employees. If the vaccine is obtained at other facilities, the cost will be borne by the Contractor, not the Government.

2.10.6. The Contractor shall immediately inform the Government when Contractor employees working in the MTF become pregnant. The Government will notify the Contractor Representative of any work hazards. For the safety of the Contractor's employee, if work hazards exist, it will be the Government's decision whether the worker can continue work in the environment.

2.10.7. Proof of subsequent immunizations required for continued employment at the MTF must be provided on the anniversary of the employee's hire date or when the option year is executed, whichever comes first.

2.11. Access to the GBS. Access to the GBS will be available to the Contractor remotely (at Government-approved locations) with the use of a government-issued Common Access Card (CAC). The Government does not require third party billing and collection services be performed in an MTF. However, to facilitate billing and collection services that require access to the GBS the Government will provide space in the MTFs in accordance with Attachment 1 – List of MTFs on a space available basis that may change without notice if the Government requires the space. Access to the GBS database for Outside the Continental United States (OCONUS) MTFs will be provided to Contractor employees assigned to Continental United States (CONUS) or other OCONUS MTFs (access to OCONUS GBS databases can typically be provided from any CONUS or OCONUS MTF location). In general, the data within a given GBS database is specific to the MTF location; however, the servers at the bases listed in Attachment 2 – Consolidated Servers, support multiple MTFs.

2.11.1. The Contractor shall only access the GBS from Government-approved locations. At no time will the Contractor or any contract employee perform services under this contract from their personal residence or other public locations without specific written authorization from the COR. The Contractor's proposal shall include all locations where they intend to perform services under this contract. The locations must be approved by the Government prior to the contract start date to ensure access to the GBS is authorized for the proposed locations. The Contractor must have a Data Sharing Agreement and Business Associate Agreement on file. Following contract start, any changes to the locations shall be mutually agreed upon by the CO, COR and the Contractor.

2.11.2. If Contract employees are assigned to work within an MTF or any other Government provided space, those employees shall only perform services directly supporting the Government requirements under this contract. If contract employees are assigned to work within an MTF, the employees shall comply with all requirements listed in this Performance Work Statement.

2.11.3. In all cases, the Contractor shall maintain segregation of duties between the employee who generates bills, and the employee who posts the payment for that bill (i.e., the same person that generates the bill cannot be the person to post payment/write-offs).

3. SECURITY

3.1. Security Requirements. Contractor personnel will have access to and/or process information requiring protection under the Privacy Act of 1974 on unclassified Automated Information Systems (AIS) under this contract. Due to this required access, contractor personnel are in non-sensitive public trust positions designated as AIS III (formally ADP) positions IAW DoD Directive 5200.2-R. In compliance with DoD Directive 5200.2-R and AFI 31-501, a National Agency Check with Inquiries (NACI) is required for all AIS III positions. The Contractor shall adhere to the provisions in these referenced publications by having all contractor personnel performing under this contract submit the appropriate forms and have a favorable outcome from the NACI investigation. All contractor personnel will schedule an appointment with the appropriate Government Unit Security Manager at the installation where services are provided and/or through the Contractor Human Resource Manager and DHA Security Manager. The Contractor shall ensure no employee has access to the information or systems required under this contract without meeting the

requirements outlined in this contract and the referenced directives, regulations, and instructions. Contractor personnel shall handle and safeguard any unclassified but sensitive information in accordance with appropriate DoW security regulations. Any security violation shall be reported immediately to the COR. No foreign nationals shall be employed under this contract without prior approval of the Government.

- 3.1.1.** The Contractor shall appoint a Security Officer to be the sole point of contact for NACI/clearance processing with the COR. The Security Officer shall ensure the following forms reach the appropriate Government Security Manager, prior to the employee's start date for services under this contract:
- 3.1.2.** SF-85P, Questionnaire for Public Trust Positions. An electronic SF-85P form can be accessed and submitted through the U.S. Office of Personnel Management web site at <http://www.opm.gov/e-qip/>. The Contractor Management Staff is required to provide the Government Unit Security Manager with two printed and signed (original signatures) hardcopies of the SF-85P. To eliminate possible re-submission of the security information, it is highly recommended that contractor personnel maintain a copy of the completed hardcopy of the SF-85P document.
- 3.1.3.** Two (2) sets of FD-258, FBI Fingerprint Cards for each contract employee. In some cases, contractor personnel may also be required to submit a Birth or Naturalization Certificate to the appropriate Security Manager.
- 3.1.4.** The Contractor shall advise contractor personnel that a positive (favorable) NACI report is needed as a condition of employment under this contract. Contractor shall make the appropriate staffing adjustments and plan accordingly for the approval process. While the Government will make every effort to expedite investigations, typical approval from the time all paperwork is received by the Government from a contract employee ranges from 30 to 60 days and may last as long as 180 days.
- 3.1.5.** At the start of the contract and each option year, The COR and/or MTF Uniform Business Office (UBO) Managers shall provide the names and phone number of the MTF's Primary Security Manager and Alternate.
- 3.1.6.** The Contractor's Security Officer shall be responsible for coordinating the initiation of the background investigation for all contract employees prior to performance following MTF and/or DHA Security protocol. They will inform both the COR and the MTF or DHA Security Manager of the new hire.
- 3.1.7.** Authorization for renewal CAC cards shall be obtained through the MTF Security Manager and/or DHA Security protocol for contractor personnel performing services in the MTF and/or remotely. All CAC cards shall be issued at local Real-Time Automated Personnel Identification System (RAPIDS) sites; the Contractor's Security Officer is responsible for identifying the nearest RAPIDS location for personnel not assigned to an MTF. All costs associated with travel to and from the RAPIDS site shall be borne by the Contractor, not the Government.
- 3.1.8.** The Contractor Representative will provide the COR with a list of current contract

employees, to include last name, first name, SSN, date of birth, work location, date favorable NACI was granted, and date data submitted for new personnel. An updated list will be provided at the start of the contract, at the start of each option year, and every time there is a change in personnel.

3.1.9. All Contractor employees requiring access to the Government unclassified computer network shall have a valid Tier 1 investigation verified through JPAS. No Contractor employee will be provided access to unclassified computer network or its inherent capabilities (i.e., internet access, electronic mail, file and print services) without a valid Tier 1 investigation. The Contractor shall be aware of and abide by all Government regulations concerning the authorized use of the Government's computer network including the restriction against using the network to recruit Government personnel or advertise job openings.

3.1.10. Disclosure of Information. In the performance of this contract, the Contractor may have access to data and information proprietary to a government agency or to another Government Contractor, or of such nature that its dissemination or use, other than as specified in this contract, would be illegal or otherwise adverse to the interests of the Government or others. The Contractor and its personnel shall not divulge, or release data or information developed or obtained under performance of this contract, except to authorized Government personnel or upon written approval of the CO. The Contractor and its Contractor personnel shall not use, disclose, or reproduce proprietary information bearing a restrictive legend, other than as specified in the contract.

3.1.11. Uses and Safeguarding of Information. Information from the secure web site is proprietary in nature when the contract number and contractor identity are associated with the direct labor hours and direct labor dollars. At no time will any data be released to the public with the Contractor name and contract number associated with the data.

4. CONTRACT REQUIREMENTS

4.1. Third Party Collections Services. The Contractor shall perform billing services in accordance with the DHA Procedures Manual, 6015.01, in its entirety to include future revisions, and all DHA applicable billing guidance (unless otherwise directed by the COR). The Contractor shall be paid a negotiated percentage of each dollar collected from third party payers and from the balance-billing of non-beneficiaries that is posted *prior to* the 121st calendar day after the date of delinquency. A debt becomes delinquent on the 31st day after the date billed. Regardless of the location where billing is performed, all data transmission fees, outside of Government locations and established communication lines, will remain the sole responsibility of the contractor (i.e., any services in support of this requirement performed outside of an MTF). To facilitate and support the TPC contract, the Contractor shall provide all personnel, supervision, training, and services necessary to perform all duties.

4.1.1. The Contractor will be required to ensure all services billable to third party payers are identified, billed, and collected or appropriately written off in accordance with DHA

guidance. The Contractor shall provide the MTF with all written documentation received on claims by scanning all hard-copy documents into Portable Document Format (PDF) format and uploading those documents to the GBS. The required documentation includes, but is not limited to copies of checks, verification of electronic payments, explanations of benefits (EOBs), demand letters, non-payment documentation, legal determinations, proof of preauthorization, pre-certifications, notes of phone conversations with insurance company personnel (including the name of the insurance representative, date/time of the phone call, and a brief summary of the discussion), and any other relevant information. Most billing data will automatically flow from other DoW automated information systems to the GBS. The contractor will contact the MTF and/or COR to seek correct billing codes from civilian, military or contracted employees (coding is performed under a separate contract) for all encounter data that does not automatically flow to the GBS. The billing contractor is not responsible for coding encounters. The Contractor shall collect all required data that did not automatically flow into GBS. For newly identified OHI, the Contractor will back-bill encounters, to the extent allowable and specified in each insurance contract or according to the Centers for Medicare and Medicaid Services (CMS) guidelines, within 30 calendar days of OHI discovery. The Contractor will be expected to request authorizations, after the fact, for care provided before back billing is performed.

4.2. Cease of Collection Activities. The Contractor shall cease collection activity on claims that are more than 120 days delinquent. Delinquent claims **MUST** be transferred back to the Government by the 120th day of delinquency. All delinquent, ignored, erroneously denied, and/or grossly underpaid claims from insurance companies must be transferred to the Office of General Counsel (OGC) no later than the 120th day of delinquency. The Contractor shall abide by DHA guidelines pertaining to delinquent debt balances, including the appropriate write off procedures. If payment on the delinquent amount is received **prior to** the 121st day of delinquency, the Contractor shall be compensated the negotiated rate per the contract. However, the Contractor shall not be compensated for any payment received after 10 business days from the time the debt was transferred back to the Government and/or OGC for debt collection. This stipulation is necessary to ensure the DHA maintains full compliance with laws and regulations pertaining to delinquent debt.

4.3. Claim Generation and Submission. The Contractor is required to work within the work item queues daily and ensure errors are resolved within **10 business days**. Work item queues and/or errors must not sit unworked past **10 business days**. The Contractor shall prepare and send inpatient claims to the third-party payers within **10 business days** following completion of the medical record and coding. The Contractor shall prepare and transmit/send outpatient claims within **10 business days** following completion of the medical record and coding. For OHI policies discovered after care was rendered, the Contractor shall back bill the insurance companies within **30 calendar days** of OHI discovery for all billable encounters that fall within the coverage timeframe. The Contractor shall collect all data necessary to manually bill. The Contractor must review additional reports and system notifications in the GBS to stay abreast of any necessary billing/collection actions.

- 4.4. Insurance Verification.** The Contractor is required to manage the paper and/or electronic DD Form 2569, Third Party Collection Program - Record of Other Health Insurance. The Contractor is required to update the GBS to reflect the collected OHI (to include updating OHI fields, scanning, and uploading any/all versions of the DD 2569) and update GBS OHI data elements. Entries may include, but not be limited to, updates to family members covered, annual deductibles, copayments, plan restrictions, plan type, medical versus pharmacy or dental carrier, and claim submission instructions. The Contractor shall perform OHI discovery using any means necessary or available to identify any information required to bill a third-party payer that was not captured on the DD 2569. When new OHI is found via receipt of an updated DD 2569 or other means of OHI discovery, this coverage shall be validated as active and billable by the Contractor (to include contacting the insurance company and determining which family members are included in the coverage) and all required data shall be entered into the GBS within **5 business days** of discovery. The DD2569 shall be scanned and uploaded into the GBS with the proper boxes checked and date added into the registration component of the GBS. The Contractor shall retrieve the necessary encounter data, ensure encounters are reviewed by a certified coder employed by the Government as a civilian, military or contracted employee (coding is performed under a separate contract), and submit these documents for back billing. All OHI shall be verified annually before the anniversary of when it was first found (even if a new DD 2569 was not completed by the patient).
- 4.5. Preauthorization and Advance Notification.** Preauthorization for care, as required by insurance companies, shall be identified and requested by the Contractor. The Contractor shall request the assistance of a Government employed civilian, military, or contract nurse assigned to the MTF when required, to provide diagnosis and treatment information. If care was provided on a non-duty day, the Contractor shall be required to request authorization, after the fact, to include sending copies of treatment notes to the insurance company. Although the Contractor may not receive advance notification when a high-dollar drug (or a drug requiring precertification) is initially dispensed, they are required to request authorization for all refills dispensed. The billing and denial of the initial dispensing will be considered a valid write-off, but all subsequent claims denied for lack of preauthorization will not be considered valid write-offs.
- 4.6. Encounter Coding/Data Review.** Ensure all encounters are billable to OHI by a certified coder employed by the Government as a civilian, military or contracted employee (coding is performed under a separate contract) prior to sending the bill to the payer. The Contractor shall review coding for encounters rejected due to errors or outpatient visits using the CPT-4 (or later versions, as applicable) for procedures and ICD-10 (or later versions, as applicable) for diagnoses and Evaluation & Management (E&M) codes for the office portion during the normal course of billing. The Contractor shall contact the MTF certified coder to correct the errors and either resubmit or cancel the bill, as appropriate.

4.6.1. The Contractor shall collect all data required for billing that does not automatically flow into the GBS. This shall include, but is not limited to, creating bills for inpatient professional charges associated with External Resource Sharing hospital care and the examples listed in paragraph 4.1.1. All billing, to include outpatient visits and ancillary services, inpatient admissions, trauma care and civilian emergencies, shall be accomplished in the GBS.

4.7. Payment Adjudication Monitoring. Claims previously billed for patients and subsequently found to have no valid OHI shall be cancelled within **3 business days** of receipt of notification to remove the accounts receivable data from the GBS. The Contractor shall immediately contact the insurance company to determine if payments received for service provided after the cancellation date need to be refunded. If a draft refund is required, the Contractor shall process the draft refund within **30 days** of notification of cancelled coverage or receipt of the refund request in accordance with COR guidance. Processing refunds include, but are not limited to, the preparation of all documentation required by the Defense Finance and Accounting Services (DFAS) and the subsequent submission of that documentation to the MTF for review and validation; however, only Government personnel (military or civilian) will transmit the documentation to DFAS. If a refund is not required, the Contractor shall add a note to each claim in the GBS to indicate a payment was received after the cancellation date, but a refund was not required.

4.8. Balance Billing. The Contractor shall not balance bill eligible DoW beneficiaries. For non-beneficiaries with OHI, the Contractor shall pursue all payments due from third party payers and the patient up to 120 calendar days after the date of delinquency.

4.9. Follow-Up/Denials Management. The Contractor shall perform active and aggressive follow-up actions for all unpaid, ignored, erroneously denied and/or grossly underpaid claims no less than every **30 calendar days** and work in tandem with any future automated and/or artificial intelligence enhancement within the GBS. The Contractor must annotate all follow-up efforts within the GBS using the standardized notes in accordance with DHA policies. The Contractor is required to transfer all delinquent, ignored, erroneously denied, and/or grossly underpaid claims from insurance companies to OGC no later than the **120th day** of delinquency with some denials requiring earlier transfer per DHA UBO policies. Any delinquent debt from the balance billing of non-beneficiaries **MUST** be transferred back to the Government by the 120th day of delinquency. The Contractor shall abide by DHA guidelines pertaining to delinquent debt balances, including the appropriate write off procedures. The Contractor shall perform denials management for all claims, regardless of amount, to ensure compliance with the timelines referenced in this PWS. To effectively manage this process, the Contractor shall utilize the Government's key performance indicators and metrics based on industry standards in denials management and report performance metrics to the COR on monthly basis. The Contractor shall identify industry ratios/indicators they intend to utilize for this contract beyond those outlined within this PWS and shall include this information in their proposal.

4.10. Unpaid Claims. The Contractor shall complete and close all valid bills no later than the 120th day of delinquency. Any amounts that remain open from insurance companies due to being ignored, erroneously denied, and/or grossly underpaid must be transferred to OGC no later than the 120th days of delinquency. All open claims from the balance billing of non-beneficiaries **MUST** be transferred back to the Government by the 120th day of delinquency.

4.11. Payment Posting and Refund Management. The Contractor shall post TPC payments and valid write-offs in the GBS. All forms of payment shall only be handled by MTF Government personnel. Typically, all payments and EOBs will be received by Government personnel; however, the Contractor shall retrieve all EOBs that are not mailed to the MTF and any missing EOBs. Government personnel will provide a copy of all checks and EOBs received to the Contractor. The Contractor shall then discern the account information associated with the EOB, annotate that account information or patient identification information on the EOB, and post payments and/or valid write-offs in the GBS within **24 hours** of receipt or by close of business the next business day. When the posting of a batch is started, it must be completed on the same business day. The Contractor shall reconcile with insurance companies those payments for which no claim could be found within the GBS. The Contractor shall dispute invalid denials with the insurance company to arrive at a resolution. Partial or reduced payments shall be posted in the GBS, and the Contractor shall continue collection efforts to recover the full amount due in accordance with insurance policy coverage. Any remaining balances shall be turned over to the Government in accordance with paragraph 4.9.

4.11.1. Payments that are posted electronically (i.e., Electronic Remittance Advice) shall be verified to ensure claims are paid properly.

4.11.2. MTF Government personnel will maintain responsibility and accountability for the deposit of collections received. Payments will be deposited no later than the next business day after postings are verified, usually by the next business day.

4.11.3. The Contractor shall verify refund requests are valid and process the draft refund transaction in the GBS within **30 days** of notification of cancelled coverage or receipt of the refund request. Unless otherwise directed by the COR, after the refund is processed in the GBS, electronically transmit all pertinent documents that support the refund and contain the information necessary to disburse the refund (such as payee, amount, reason for refund, EOB, etc.) to DHA MTFs personnel via GBS for processing through DFAS.

4.11.4. Program Support. Market the TPC program to patients and train MTF staff **quarterly** on their responsibilities in support of the TPC program; Contractor must provide COR, a summary report on the Contractor's requirement to train TPC program responsibilities to appropriate MTF.

All billing and collection information shall remain the property of the U.S. Government. The Government shall retain unlimited use and ownership of all billing information, data, and data/technical rights for any upgrades and/or modifications to Government systems, upon completion and/or termination of this contract. All tools developed by the Contractor in support of this requirement, (i.e. GBS ad hoc reports, Microsoft Word, Access, Excel, PowerPoint, or other software developed databases) shall become and remain the property of the Government and shall have no effect or impact on monies owed to the Contractor by the Government. Access to the GBS will be available to the Contractor with the use of a government issued CAC – reference paragraph 3.1. The Contractor shall collaborate with the COR and any other required personnel (i.e., MTF Systems and Security Officers) to obtain access to the GBS and any other Government applications (such as required existing legacy systems still in place during the period of performance) as required to execute this contract.

4.11.5. Audit/Compliance. The Government shall have the right to audit any data produced or any function performed by the Contractor at any time and shall have access to any Contractor owned, managed, or otherwise operated facilities where services are performed under this contract without advance notice.

The Contractor may be asked to participate in meetings to facilitate discussions providing input as billing and collection subject matter experts on behalf of the DHA.

5. QUALITY CONTROL PLAN (QCP)

5.1. QCP. The Contractor shall establish and maintain a complete Quality Control Plan. The COR shall review the Quality Control Plan annually. The plan shall include procedures to implement all requirements of the contract to include the in-processing submittals for all contract personnel and the following:

- 5.1.1.** A compliance plan for TPC in accordance with DHA UBO guidelines.
- 5.1.2.** An inspection system covering all the services required under this contract. The Contractor shall specify the areas to be inspected on either a scheduled or unscheduled basis, how often inspection shall be accomplished and documented, and identify the title of the individual(s) who shall perform the inspection. The Contractor shall maintain a record of all inspections conducted by the Contractor and corrective actions taken. This documentation shall be made available to the Contracting Officer (CO)/COR upon request during the term of the contract. The methods of identifying and preventing defects in the quality of services performed before the level of performance becomes unacceptable.
- 5.1.3.** Written standards (developed by the Contractor) for documentation and fraud prevention for third party collections.
- 5.1.4.** The policy and procedures to ensure all contractor personnel have the required TPC skills and training initially and annually. The plan should also identify the individual(s) performing the assessment.
- 5.1.5.** The Contractor shall establish and maintain an internal policy and procedures checklist for conducting TPC quality control checks to ensure work is free of grammatical, typographical, punctuation, and spelling errors and is properly formatted prior to delivery of products.
- 5.1.6. Customer Satisfaction.** The Contractor shall have a customer satisfaction system outlining internal controls and procedures to measure DHA customer satisfaction. The process shall outline methods and frequency of customer interactions, reporting processes, and problem resolution procedures. A satisfaction measurement of patient satisfaction is not required or authorized.

6. POST AWARD AND INITIAL CONTRACT PERFORMANCE

6.1. Post Award. The Government will host a post award meeting with the Contractor within

10 business days after contract award. The CO shall contact the COR and the Contractor to schedule this meeting. Telecon/teleconference is permissible for the post-award meeting. The purpose of the post-award meeting is to introduce the Contractor to the Government representatives (COR) and go over the contract. Takeaways from the meeting should be an exchange of contact information; a timeline of when direct support will start, if not yet started; and an understanding by all parties of all the requirements to be met by the Contractor and the support the Government will provide for the Contractor to perform the requirements safely and efficiently.

- 6.2. Business Process.** The Contractor shall provide their business process with their proposal to include an organizational management structure and specific business approach to the requirements listed throughout this PWS. The proposal shall clearly define roles, responsibilities, subcontractors and/or business partners to ensure effective and efficient TPC services. In addition, the Contractor shall demonstrate a comprehensive process for revenue cycle management that specifically details their approach to the medical claims processing continuum. The Contractor shall notify the COR of any significant changes to their business process during the contract. All contractor instruction given to MTFs that change level of effort required of MTF staff to facilitate execution of the requirements of this PWS shall go through COR for process review.
- 6.3. Human Resource Management Plan.** The Contractor shall have in place a Human Resource Management Plan to ensure qualified candidates are identified, screened, placed, monitored, trained, and retained to fulfill service requirements and minimize personnel turnover under this contract. The Contractor shall accomplish the assigned work by employing and utilizing qualified personnel with appropriate combinations of education experience, knowledge, training, abilities and skills. The plan shall include quality assurance and productivity measurements that demonstrate internal controls for workload management that directly support attainment of acceptable output standards and the efficient allocation of resources for performance in accordance with this PWS. These systems and controls for recording and monitoring workload shall be capable of providing historical and in- process workload data.
- 6.4. Medical Expense and Performance Reporting System (MEPRS).** MEPRS is the DoD accounting system for financial, personnel, and workload data within MTFs. The Defense Medical Human Resources System-internet (DMHRSi) is the interface used to report hours worked in the MTF. Per DHA-PM 6010.13, *Medical Expense and Performance Reporting System for Fixed Military Medical and Dental Treatment Facilities*, when working within an MTF, the Contractor is required to submit hours worked bi-weekly through DMHRSi to their assigned MTF.
- 6.5. Invoices.** A monthly invoice listing the total collections per MTF and the amount due the Contractor, shall be submitted on a Microsoft Excel spreadsheet on **5th business day** the following month (editable version). The invoice will be processed for payment after billed amounts are verified by the COR.

7. TRANSITION/CONTINGENCY PLANS

- 7.1. Phase-in Plan.** The Contractor shall include a Phase-in Plan as part of their proposal that

describes in detail what actions the Contractor will take to ensure their employees are prepared to begin performance for all services required under this PWS and the process to minimize disruption in services throughout the transition. The Contractor shall attend a post award orientation meeting within **10 business days** after contract award to review requirements and implementation of the Contractor's transition plan.

- 7.2. Phase-out Plan.** The Contractor shall include a phase-out plan as part of their proposal that describes in detail what actions the Contractor will take to ensure a smooth and successful transition of operations between this contract and the next contract provider. Assistance shall include but is not limited to detailing current requirements, suggesting milestones and suggesting necessary actions for the last **30 business days** of the contract; this may include formal coordination with Government staff or contractor successor staff as needed. The plan should illustrate a process that would allow the Contractor to "turn over Government claims data" within **30 calendar days** (in a mutually agreed upon format) whereby the Government could continue uninterrupted billing and collections services. It shall also include providing any information pertinent to the daily activities as outlined in the PWS such as duties, responsibilities, data, metrics, or requirements that are critical for ensuring continued operations.
- 7.3. Contingency Plan.** The Contractor shall maintain a contingency plan to ensure continuity of operations for all services required under this PWS. In the event of staffing shortages, equipment or communication failures, or other delays associated with the performance of TPC services under this contract, on the part of the Contractor, a contingency plan shall be implemented to continue collection and billing services at the MTF. It shall describe how the Contractor will continue to meet production standards during the repair or replacement of faulty equipment. If the contingency plan fails during an actual failure of operations, the Government will require the contractor to modify the plan within **5 business days**, in addition to any other re-performance associated with the failure of the plan.

8. CONTRACTOR QUALIFICATIONS

- 8.1. Experience and Requirements.** The Contractor shall have a demonstrated, in-depth knowledge of best practices associated with Government healthcare and the health insurance industry as they relate to the performance of all services described in this PWS at multiple MTFs in a Government or commercial setting. The Contractor shall have experience performing revenue cycle back-end functions for claims submissions, insurance identification and verification, accounts receivables management, denials and appeals management, and collections follow-up within the healthcare industry. In addition, the Contractor shall have a working knowledge of Ambulatory Procedure Groupings (APGs), Diagnosis Related Groupings (DRGs), International Classification of Diseases-Version 11 (ICD-11) coding, electronic billing and collection procedures (to include future versions as appropriate) and itemized Revenue Cycle billing. In addition, the Contractor shall have legal assistance, a Registered Nurse, and Registered Information Health Care Administrators on staff or immediately available to facilitate and expedite collection processes.
- 8.2. Personnel Qualifications.** The Contractor shall employ a staff able to read and interpret medical documentation and understand medical terminology to correctly bill both outpatient and inpatient services. In addition, the Contractor shall employ a staff with at least two years of experience in the operation of personal computers and familiarity with common office software to include, but

not limited to, Microsoft Excel, Microsoft Word, Microsoft PowerPoint, Microsoft Access, and electronic mail systems.

8.2.1. At a minimum, at any location where services are performed under this contract (MTF and/or external location), there shall be one or more contractor employees who possess either two years of experience in medical billing and collections or have completed a medical biller certification program. The Government considers experience acceptable if it was performed within the DoW or in a commercial healthcare organization; however, it must be specific to medical billing and collection services. At a minimum, the Contractor's management team shall include the following key personnel who meet the requisite experience requirements as identified below:

- Contractor Representative – The Contractor Representative shall have a minimum of five (5) years' experience performing all services described in this PWS at multiple MTFs in a Government or commercial setting.
- Billing and Collection Manager – The Billing and Collection Manager shall have a minimum of five years' experience supervising medical billers and support staff at multiple locations. This individual shall also have a minimum of five years' experience simultaneously dealing with medical insurance companies across multiple markets and the practical application of laws and regulations governing medical billing and collection practices nationwide.
- Human Resource Manager – The Human Resource Manager shall possess a Senior Professional in Human Resources (SPHR) or similar certification and have a minimum of five years' experience managing medical administrative personnel.
- Training Manager – The Training Manager shall have a minimum of five years' experience training medical administrative personnel engaged in billing and collections operations.

8.3. Certification Statement. The Contractor must provide a signed certification statement that certifies personnel performing under this PWS meet the requirements within paragraphs 8.2.1. The signed certification statement is required within *ten calendar days* following contract award. *Monthly* staff rosters shall highlight those employees who possess the qualifications listed under paragraphs 8.2.1.

8.4. Government Furnished Training. The Government will provide initial training to the Contractors' education and training department (no more than two persons) for subsequent Contractor training ("train the trainer") to their employees on all Government furnished software applications systems to include, but not limited to, the GBS and required legacy systems. The Contractors shall be responsible for all future training necessary to perform the work as defined in this PWS. Training shall not hamper the quantity, quality, or timeliness of daily work requirements.

8.5. Continuing Training. Training of new Contractor employees will be the Contractor's

responsibility. Training shall not hamper the quantity, quality or timeliness of daily work requirements. Contractors are authorized and encouraged to participate in UBO training provided by the DHA at no cost to the Contractor.

- 8.6. Identification of Contractor Employees.** The Contractor shall require Contractor employees to identify themselves as Contractor personnel by introducing themselves or being introduced as Contractor personnel and displaying distinguishing badges or other visible identification for meetings with Government personnel. In addition, the Contractor shall require Contractor personnel to appropriately identify themselves as Contractor employees in telephone conversations and in formal and informal written correspondence.

9. CONTINUITY OF SERVICES

- 9.1. Continuity of Services.** Contractors shall ensure continuity of operations, without lapses, throughout the term of this contract. The Contractor shall coordinate with the CO/COR any anticipated changes in operations which might affect the performance of this contract.
- 9.2. Subcontracting.** The Government will not preclude Contractors from subcontracting or partnering for specific services in support of this PWS. If the Contractor elects to subcontract or partner with another company, the Contractor shall provide the names of subcontracted/partnering companies and employees who directly support this effort. In addition, subcontract or partnering company employees shall meet and comply with the same requirements (described above) as the primary Contractor.
- 9.2.1.** The Contractor shall adhere to DoDI 8582.01, *Security of Unclassified DoD Information on Non-DoD Information Systems*, for all DoD information that interface with the Contractor's (or subcontractor's) servers in support of this contract.

10. ADDITIONAL ADMINISTRATIVE CONSIDERATIONS

- 10.1. Contract Management.** Management of this contract will be performed through the COR. The Contractor is responsible for coordinating all contract related matters with the COR. This includes ensuring requirements specified in this contract are available when needed and identifying any problems that may affect TPC services.
- 10.2. Inspection and Acceptance.** The COR will manage PWS tasks and deliverables to ensure that all PWS requirements are completed. The COR shall have seven business days to review deliverables and make comments. The Contractor shall have five business days to make corrections. Upon receipt of all final deliverables, the COR shall have five business days to either accept or reject the corrected deliverable.
- 10.2.1. Requests for Documentation and Meeting.** The Contractor shall accommodate the need for timely cooperation in the overall project and understand that time is of the essence, particularly regarding requests for documentation and informational meetings. The Contractor shall coordinate actively and responsively with Government personnel. The Contractor shall coordinate and be cooperative with other contractors. Failure to coordinate precludes effective performance under this PWS.

11. PERSONALLY IDENTIFIABLE INFORMATION (PII) AND PROTECTED HEALTH INFORMATION (PHI)

- 11.1. Safeguards.** The Contractor shall establish appropriate administrative, technical, and physical safeguards to protect all Government data. The Contractor shall also ensure the confidentiality, integrity, and availability of Government data in compliance with all applicable laws and regulations, including data breach reporting and response requirements, in accordance with DFAR Subpart 224.1 (Protection of Individual Privacy), which incorporates by reference DoD 5400.11, "DoD Privacy Program," May 8, 2007, and DoD 5400.11-R, "DoD Privacy Program." The Contractor shall also comply with federal laws relating to freedom of information and records management.
- 11.2. Health Insurance Portability and Accountability Act of 1996 (HIPAA):** The Contractor shall comply with all requirements of the HIPAA (Pub. L. 104-191) and all subsequent revisions, as implemented by the HIPAA Privacy and Security Rules codified at 45 C.F.R. Parts 160 and 164, and as further implemented within the Military Health System (MHS) by DoD 6025.18-R, "DoD Health Information Privacy Regulation," January 24, 2003, and DoD 8580.02-R, "DoD Health Information Security Regulation," July 12, 2007. The Contractor shall also comply with all applicable HIPAA-related rules and regulations as they are published and as further defined by later-occurring Government requirements and DoD guidance, including current and forthcoming DoD guidance implementing applicable amendments under the American Recovery and Reinvestment Act of 2009 (ARRA). If HIPAA violation occurs, contractor will contact COR immediately after initial notification of report of violation.
- 11.3. Breach Response.** The Contractor shall adhere to the reporting and response requirements set forth in the Office of the Secretary of Defense (OSD) Memorandum 1504-07, "Safeguarding Against and Responding to the Breach of Personally Identifiable Information," June 5, 2009; DoW 5400.11-R, and applicable DHA Privacy Office guidance, including current and forthcoming DoW guidance on ARRA breach notification requirements, available at: <https://health.mil/military-health-topics/privacy-and-civil-liberties/breaches-of-PII-and-PHI>.
- 11.4.** See Appendix B Business Associate Agreement.

12. PERFORMANCE OBJECTIVES

- 12.1. Services Summary (SS).** The Contractor services requirements are summarized as performance objectives that relate directly to mission essential items. The performance threshold briefly describes the minimum acceptable levels of service for each requirement. These thresholds are critical to mission success.
- 12.2. Monthly Status Reports.** Monthly status reports are due to the COR and the MTFs by the *15th calendar day* of the following month. For each performance objective, MTFs shall receive reports specific to their operations whereas the COR will receive a report that includes Government totals and the MTF breakdown. The format and method of delivery shall be mutually agreed upon by the COR and the Contractor.

12.3. Services Summary Table.

TABLE 1 - Services Summary		
Performance Objectives	PWS Paragraph	Performance Threshold
Revenue Cycle Operations		
# 1 <u>Claims Generation and Submission</u> The Contractor shall: Work within the work item queues daily and ensure errors are resolved within 10 business days . Prepare and transmit/send inpatient and outpatient claims to the third-party payers within 10 business days following completion of the medical record and coding.	4.3	95% of actions accomplished within the specified timeframe for each MTF
# 2 <u>OHI Verification</u> The Contractor shall: Within 5 business days of the electronic discovery of new OHI enter the OHI data into the GBS as appropriate. Re-verify OHI annually before the anniversary of when it was first discovered.	4.4	95% of actions accomplished within the specified timeframe for each MTF
# 3 <u>Payment Posting and Refund Management</u> The Contractor shall: Post payments in the GBS within 24 hours of receipt (or by COB of the second business day). Verify draft refund requests and process the refund transaction in the GBS within 30 days of receipt.	4.7 4.10	95% of actions accomplished within the specified timeframe for each MTF
# 4 <u>Follow-Up/Denials Management</u> The Contractor shall: Perform active and aggressive follow-up actions for all unpaid, ignored, erroneously denied and/or grossly underpaid claims at least every 30 calendar days . Transfer all delinquent, ignored, erroneously denied, and/or grossly underpaid claims from	4.2 4.8 4.9	95% of actions accomplished within the specified timeframe for each MTF

insurance companies to OGC no later than the <i>120th day</i> of delinquency.		
Transfer all delinquent debt from the balance billing of non-beneficiaries back to the Government by the <i>120th day</i> of delinquency.		
# 5 <u>Unpaid Claims</u> The Contractor shall: Complete and close all valid bills no later than the <i>120th day</i> of delinquency.	4.10	95% of actions accomplished within the specified timeframe for each MTF
# 6 <u>Training Requirement</u> The Contractor shall: Market the TPC program to patients and train MTF staff <i>quarterly</i> on their responsibilities in support of the TPC program.	4.11.4	95% of actions accomplished within the specified timeframe for each MTF

13. GOVERNMENT FURNISHED PROPERTY AND SERVICES

- 13.1. General.** Government Furnished Equipment (GFE) shall be provided for the Contractors. Contractors physically located in an MTF will receive GFE from the MTF. Remote Contractors will receive GFE from the COR in accordance with DHA policies.
- 13.1.1. Equipment.** Only contractor employees physically located at the MTF(s) shall have joint use of all equipment and furniture necessary for performing services required by this contract. The MTF will provide furnished office space sufficient to provide a working environment conducive to effective performance of the tasks required under this contract. The space provided within an MTF shall be at the sole direction of the MTF Commander.
- 13.1.2. Inventory.** Inventory of all GFE shall be performed within 10 business days of contract start, and as required. Both the Contractor employee and Government will be responsible for taking inventory.
- 13.1.3. Supplies.** The Contractor shall establish procedures with the COR and respective MTF for requests of all required Government forms (exception: Contractor shall purchase billing forms such as the UB-04, HCFA 1500, etc.) and documents used in the performance of TPC services as specified in this PWS. All records, files, documents, and work papers provided by the Government remain Government property.
- 13.1.4. Administrative Support.** In addition to GFE the Government shall provide telephone lines and defense switching network (DSN) lines, access to copying machines, fax machines, medical library, as well as Government computer systems, and office space for contractors working in an MTF. These support items shall be used only for official Government business in performance of TPC requirements under this contract.

13.1.5. Housekeeping. The Contractor shall comply with and not interfere with Government provided routine housekeeping at the MTF.

13.1.6. MTF's Facility Orientation (Safety Material). The Contractor shall ensure that all Facility Orientation Safety Material and post-tests the Government provides for contractor personnel at the MTF are completed and ensure that contractor employees physically located at MTFs comply with the identified material prior to performance.

13.1.7. Reporting Equipment Failure. During normal hours of operation (7:30 a.m. to 4:30 p.m. local time), the Contractor shall notify the COR of any equipment failures.

14. DELIVERABLES

14.1. Aging of Accounts Receivable. In accordance with the FMR Vol 4, Chapter 3, accounts are considered "current" until they reach the due date. The "due date" is the 30th day following the date billed. If payment has not been made by the due date, the debt will be reported as delinquent beginning the first calendar day after the due date. Delinquent accounts will be aged and reported according to the following categories: Current, 1-30 days, 31- 60 days, 61- 90 days, 91-180 days, 181-365 days, 1-2 years, 2-6 years, 6-10 years, and greater than 10 years. A summary aging schedule will be provided in the notes of the Government financial statements and reconciled to the general ledger. For example, the aging of receivables schedule will be reconciled with the designated accounts receivable general ledger account. Specific guidance on aging accounts receivable is found in FMR Volume 6B, Chapter 10, "Notes to the Financial Statements" or FMR Vol 4 Chapter 3.

14.1.1. Aging Activity Report (A/R). The Contractor shall conduct *monthly* analysis of the aging A/R according to the aging categories listed in paragraph 14.1. The contractor shall provide justification for any barriers/issues regarding collection activities and provide a corrective action plan to close all accounts and/or transfer to OGC prior to the 120th day of the delinquency deadline. The Contractor shall exhaust all internal options and make every attempt to collect and properly close all accounts before the 120th day of the delinquency deadline.

14.2. DD 2570 Report. The Contractor shall provide a DD 2570, Third Party Collections Program Report on Program Results for TPC inpatient and outpatient collections only on a quarterly basis. The DD 2570 report shall be generated from the GBS and provided to the COR and individual MTFs. These reports are due to the COR and the MTFs by the *5th calendar day* following the end of the fiscal year quarter.

14.3. Monthly Invoice. Monthly invoices will be submitted (in a format specified by the COR) to the COR by the *5th business day* following the end of each month. Upon verification, the COR will instruct the Contractor to submit the 2-in-1 invoice through Wide Area Workflow. The invoice will be accepted and processed for payment as soon as possible.

APPENDIX A

ACRONYMS

Acronyms and Definitions	
ACRONYM	DEFINITIONS
AIS	Automated Information System
APG	Ambulatory Procedure Visit
APV	Ambulatory Procedure Visit: An outpatient surgery procedure.
A/R	Activity Report
ARRA	American Recovery and Reinvestment Act of 2009
BLS	Basic Life Support
CAC	Common Access Card
CCE	Coding and Compliance Editor – Back-end automated coding edit tool
CDC	Centers for Disease Control and Prevention
CMS	Centers for Medicare and Medicaid Services: Formerly known as HCFA
CONUS	Continental United States
CO	Contracting Officer: Appointed officer with authority obligate on behalf of the gov't
COR	Contracting Officer Representative:
CPT	Current Procedural Terminology: Used for coding medical/surgical services
DHA	Defense Health Agency
DFAS	Defense Finance and Accounting Services
DMHRSi	Defense Medical Human Resources System-internet
DMIS ID	Defense Medical Information System Identification
DoW	Department of War
DRG	Diagnosis Related Group
E&M	Evaluation & Management
EOB	Explanation of Benefits
FSO	Facility Security Officer
GBS	Government Billing Solution: Program used for medical billing/collection activities
HIPAA	Health Insurance Portability and Accountability Act of 1996
IAW	In Accordance With
ICD	International Classification of Diseases.
JKO	Joint Knowledge Online
MEPRS	Medical Expense and Performance Reporting System
MTF	Military Treatment Facility: Refers to a military medical treatment facility
NACI	National Agency Check with Inquiries
OCONUS	Outside the Continental United States
OGC	Office of General Counsel
OHI	Other Health Insurance
PDF	Portable Document Format
PWS	Performance Work Statement

QCP	Quality Control Plan: Gov't actions taken to assure services meet PWS requirements
RAPIDS	Real-Time Automated Personnel Identification System
SS	Services Summary
SSN	Social Security Number
TPC	Third Party Collections
UBO	Uniform Business Office

APPENDIX B

Department of War (DoW) Business Associate Agreement (BAA)

Introduction

In accordance with 45 CFR §§164.502(e)(2), 164.504(e); the Health Information Technology for Economic and Clinical Health (HITECH) Act; and paragraph 3.3.c. of Department of Defense Manual (DoDM) 6025.18, “Implementation of the Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule in DoD Health Care Programs,” March 13, 2019, and Chapter 1, Section 5 of the TRICARE Operations Manual, this document serves as a Business Associate Agreement (BAA) between the executing parties for purposes of the Health Insurance Portability and Accountability Act (HIPAA) as implemented by the HIPAA Rules and Department of Defense (DoD) HIPAA Issuances (as defined below). The parties are a DoD Component, acting as a HIPAA Covered Entity, and a Business Associate (i.e., a DoD Contractor creates, receives, maintains, and/or transmits protected health information (PHI) for the purpose of performing covered functions on behalf of the DoD Component). The HIPAA Rules (as defined below) require BAAs between covered entities and business associates. As such, this BAA implements and incorporates the applicable DoD HIPAA Issuances (including DoDI 6025.18 and the authorities incorporated therein) and provides the Business Associate requirements which apply to the relevant Business Associates contract or other agreement between the parties.

- (a) **Catchall Definition::** Except as otherwise provided in this BAA, the following terms used in this BAA shall have the same meaning as those terms in the DoD HIPAA Issuances : Data Aggregation, Designated Record Set, Disclosure, Health Care Operations, Individual, Minimum Necessary, Notice of Privacy Practices (NoPP), PHI,, Required by Law, Secretary of HHS, Security Incident, Subcontractor, Unsecured Protected Health Information, and Use.

- (b) **Specific definitions:**

Agreement means this BAA together with the documents and/or other arrangements under which the Business Associate signatory performs services involving access to PHI on behalf of the DoD component signatory.

Breach means the loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where: (1) a person other than an authorized user accesses or potentially accesses personally identifiable information; or (2) an authorized user accesses or potentially accesses Personally Identifiable Information (PII) for an other than authorized purpose. The foregoing definition is based on the definition of breach in Office of Management and Budget (OMB) Memorandum M-17-12.

Business Associate shall generally have the same meaning as the term “Business Associate” in the DoD HIPAA Issuances, and in reference to this BAA, shall mean *[insert name*

of the non-federal Business Associate entity/signatory to this BAA].

Covered Entity shall generally have the same meaning as the term “covered entity” in the DoD HIPAA Issuances, and in reference to this BAA, shall mean *[insert name of the DoD Component entity/DoD signatory to this BAA]*.

Covered Functions are functions of a covered entity, the performance of which makes the entity a health plan or health care provider as outlined in DoDM 6025.18.

Defense Health Agency (DHA) Privacy Office means the DHA Privacy and Civil Liberties Office, with the responsibilities and authorities as outlined in DoDM 6025.18. The Chief of the DHA Privacy Office is the HIPAA Privacy and Security Officer for DHA.

DoD HIPAA Issuances means all DoD issuances implementing the HIPAA Rules in the DoD Military Health System (MHS). These issuances include DoDM 6025.18 (2019), Department of Defense Instruction (DoDI) 6025.18, “Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule Compliance in DoD Health Care Programs,” March 13, 2019; and DoDI 8580.02, “Security of Individually Identifiable Health Information in DoD Health Care Programs,” August 12, 2015.

DoD Privacy Program Issuances means the current DoD issuances implementing within DoD the Privacy Act and certain privacy-related authorities, as identified by DHA Privacy Office Guidance. These issuances are DoDI 5400.11, “DoD Privacy and Civil Liberties Programs,” January 29, 2019, DoDM 5400.11, Volume 2 “DoD Privacy and Civil Liberties Programs: Breach Preparedness and Response Plan,” May 6, 2021, and DoD 5400.11-R, “Department of Defense Privacy Program,” May 14, 2007. These issuances are available on the Washington Headquarters Services DoD Directives website (<https://www.esd.whs.mil/DD/>) or upon request.

Department of Health and Human Services (HHS) Breach means a breach that satisfies the HIPAA Breach Rule definition of breach found in 45 CFR §164.402.

HIPAA Rules means the regulations issued by HHS pursuant to its authority to issue regulations on health information privacy, as provided by Section 264(c) of HIPAA. The HIPAA Rules, as amended by the Omnibus Final Rule, include the HIPAA Privacy Rule, the HIPAA Breach Rule, the HIPAA Security Rule, and the HIPAA Enforcement Rule.

I. Obligations and Activities of the Business Associate

(a) The Business Associate shall not use or disclose PHI other than as permitted or required by the Agreement or as required by law.

(b) The Business Associate shall use appropriate safeguards, and comply with the HIPAA Rules and DoD HIPAA Issuances incorporated by reference in this document Issuances with respect to PHI, to prevent use or disclosure of PHI other than as provided for by the Agreement.

(c) The Business Associate shall report to the Covered Entity any breach of which it

becomes aware, and shall proceed with breach response steps as required by Part V of this agreement. With respect to electronic PHI, the Business Associate shall also respond to any security incident of which it becomes aware in accordance with any cybersecurity provisions of the Agreement. If at any point the Business Associate becomes aware that a security incident involves a breach, the Business Associate shall immediately initiate breach response as required by Part V of this BAA.

(d) In accordance with DoDM 6025.18, paragraph 3.3.c.(3)(b)4, 45 CFR §164.502(e)(1)(ii) and §164.308(b)(2), the Business Associate shall ensure that any (and all) subcontractors that create, receive, maintain, or transmit PHI on behalf of the Business Associate agree to the same restrictions, conditions, and requirements that apply to the Business Associate with respect to PHI, specifically the responsibilities laid out in the DoD HIPAA Issuances incorporated by reference in this agreement. PHI.

(e) The business associate may disclose PHI to a business associate that is a subcontractor and may allow the subcontractor to create, receive, maintain, or transmit PHI on its behalf, if the business associate obtains satisfactory assurances, in accordance with DoDM 6025.18, paragraph 4.5.e.(1), that the subcontractor will appropriately safeguard the information.

(f) The Business Associate shall make available PHI in a Designated Record Set, to the Covered Entity or, as directed by the Covered Entity, to an Individual, as necessary to satisfy the Covered Entity obligations under 45 CFR §164.524 and DoDM 6025.18, paragraph 5.3.c.

(g) The Business Associate shall make any amendment(s) to PHI in a Designated Record Set as directed or agreed to by the Covered Entity pursuant to 45 CFR § and DoDM 6025.18, paragraph 5.4.

(h) The Business Associate shall maintain and make available the information required to provide an accounting of disclosures to the Covered Entity or an individual as necessary to satisfy the Covered Entity's obligations under 45 CFR §164.528 and DoDM 6025.18, paragraph 5.5.

(i) To the extent the Business Associate is to carry out one or more of Covered Entity's obligation(s) under the HIPAA Privacy Rule and DoDM 6025.18, the Business Associate shall comply with the requirements of the HIPAA Privacy Rule and DoDM 6025.18, that apply to the Covered Entity in the performance of such obligation(s); and

(j) The Business Associate shall make its internal practices, books, and records relating to the use and disclosure of PHI from or created or received by the Business Associate on behalf of, the DoD Component available to the Secretary of HHS and to the Director, DHA, or their designee for purposes of determining compliance with the HIPAA Rules.

II. Permitted Uses and Disclosures by Business Associate

(a) The Business Associate may only use or disclose PHI as necessary to perform the services set forth in the Agreement or as required by law. The Business Associate is not permitted

to de-identify PHI, nor is it permitted to use or disclose de-identified PHI, except as provided by the Agreement or directed by the Covered Entity with written approval from DHA's HIPAA Privacy Officer.

(b) The Business Associate agrees to use, disclose, and request PHI only in accordance with the HIPAA Privacy Rule "minimum necessary" standard and corresponding DHA policies and procedures as stated in the DoD HIPAA Issuances.

(c) The Business Associate shall not use or disclose PHI in a manner that would violate the DoD HIPAA Issuances if done by the Covered Entity.

(d) Except as otherwise limited in the Agreement, the Business Associate may use PHI for the proper management and administration of the Business Associate or to carry out the legal responsibilities of the Business Associate. The foregoing authority to use PHI does not apply to disclosure of PHI, which is covered in the next paragraph.

(e) Except as otherwise limited in the Agreement, the Business Associate may disclose PHI for the proper management and administration of the Business Associate or to carry out the legal responsibilities of the Business Associate, provided that disclosures are required by law, or the Business Associate obtains reasonable assurances from the person to whom the PHI is disclosed that it will remain confidential and used or further disclosed only as required by law or for the purposes for which it was disclosed to the person, and the person notifies the Business Associate of any instances of which it is aware in which the confidentiality of the information has been breached.

(f) Except as otherwise limited in the Agreement, the Business Associate may use PHI to provide Data Aggregation services relating to the Covered Entity's health care operations.

III. Provisions for Covered Entity to Inform Business Associate of Privacy Practices and Restrictions

(a) The Covered Entity shall provide the Business Associate with NoPP that the Covered Entity produces in accordance with 45 CFR §164.520 and DoDM 6025.18, paragraph 5.1.

(b) The Covered Entity shall notify the Business Associate of any changes in, or revocation of, the permission by an Individual to use or disclose his or her PHI, to the extent that such changes affect the Business Associate's use or disclosure of PHI.

(c) The Covered Entity shall notify the Business Associate of any restriction on the use or disclosure of PHI that the Covered Entity has agreed to or is required to abide by under 45 CFR §164.522 and DoDM 6025.18, paragraph 5.2, to the extent that such changes may affect the Business Associate's use or disclosure of PHI.

IV. Permissible Requests by Covered Entity

The Covered Entity shall not request the Business Associate to use or disclose PHI in any manner that would not be permissible under the HIPAA Privacy Rule or any applicable Federal

regulations (including without limitation, DoD HIPAA Issuances) if done by the Covered Entity.

V. Breach Response

(a) In general.

In the event of a breach of PII/PHI held by the Business Associate, the Business Associate shall follow the breach response requirements set forth in this Part V, which is designed to satisfy both the Privacy Act and HIPAA breach response requirements, as applicable. If a breach involves PII without PHI, then the Business Associate shall comply with DoD Privacy Program Issuances breach response requirements only. If a breach involves PHI (a subset of PII), then the Business Associate shall comply with DoD Privacy Program Issuances breach response requirements. A breach involving PHI may or may not constitute a HHS Breach. If a breach is not an HHS Breach, then the Business Associate has no HIPAA breach response obligations. In such cases, the Business Associate must still comply with breach response requirements under the DoD Privacy Program Issuances.

If the DHA Privacy Office determines that a breach is an HHS Breach, then the Business Associate shall comply with both the HIPAA Breach Rule and DoD Privacy Program Issuances, as directed by the DHA Privacy Office. If the DHA Privacy Office determines that the breach does not constitute an HHS Breach, then the Business Associate shall comply with DoD Privacy Program Issuances. The following provisions of Part V set forth the Business Associate's Privacy Act and HIPAA breach response requirements for all breaches, including but not limited to HHS breaches.

In general, for breach response, the Business Associate shall report the breach to the Covered Entity. Such breach shall be reported to the DHA Privacy Office within 24 hours at 703-275-6363 or dha.ncr.pcl.mbx.pii-breach@health.mil. If such breach is a cybersecurity incident, an incident involving damage to, protection of, and restoration of computers, electronic communications systems, electronic communications services, wire communication, electronic communication, including information contained therein, ensuring the availability, integrity, authentication, confidentiality, and nonrepudiation of data, as defined in Committee on National Security Systems Instruction (CNSSI) 4009 <https://www.cnss.gov/CNSS/issuances/Instructions.cfm>, the discovering party shall report the breach to the DHA NIWC CSSP Watchdesk (by dialing 1.866.786.4432 Cybersecurity and Infrastructure Security Agency potential-CERT) within one hour of the potential cybersecurity incident. The DHA NIWC CSSP Watchdesk reports, and report to USCYBERCOM within 48 hours of being notified of the occurrence of a breach, and complete the breach response actions as required by DHA guidance <https://health.mil/Military-Health-Topics/Privacy-and-Civil-Liberties/Breaches-of-PII-and-PHI?type=Policies#RefFeed>.

The Business Associate is deemed to have discovered a breach as of the first day a breach (suspected or confirmed) is known, or by exercising reasonable diligence would have been known, to any person (other than the person committing it) who is an employee, officer or other agent of the Business Associate.

The Business Associate shall submit a report to the U.S. Computer Emergency Readiness Team (US-CERT), using the US-CERT report online form at <https://us-cert.cisa.gov/forms/report>. Before submission to US-CERT,

the Business Associate shall save a copy of the on-line report. After submission, the Business Associate shall record the US-CERT Reporting Number. Although only limited information about the breach may be available as of the one-hour deadline for submission, the Business Associate shall submit the US-CERT report by the deadline. The Business Associate shall e-mail updated information to the Covered Entity as it is obtained. The Business Associate shall provide a copy of the initial or updated US-CERT report to the DHA Privacy Office. Business Associate general questions about US-CERT reporting shall be directed to the DHA Privacy Office not the US-CERT office.

Additionally, the Business Associate will send to the DHA Privacy Office a completed Breach Report Form Report DD 2959 at <https://www.esd.whs.mil/Portals/54/Documents/DD/forms/dd/dd2959.pdf>. Encryption is not required, because Breach Report Forms must not contain PII/PHI.

If multiple individuals are affected by a single event or related set of events, then a single reportable breach may be deemed to have occurred, depending on the circumstances. The Business Associate shall inform the DHA Privacy Office as soon as possible if it believes that a “single event” breach response is appropriate. The DHA Privacy Office will determine how the Business Associate shall proceed and, if appropriate, consolidate separately reported breaches for purposes of Business Associate report updates, individual notification, and mitigation.

When an initially submitted Breach Report Form is incomplete or incorrect due to unavailable information, or when significant developments require an update, the Business Associate shall submit a revised form or forms, stating the updated status and previous report date(s) and showing any revisions or additions by denoting “UPDATE.” Examples of updated information the Business Associate shall report include but are not limited to confirmation on the exact data elements involved, the root cause of the incident, and any mitigation actions, including sanctions, training, incident containment, follow-up, etc. The Business Associate shall submit these report updates promptly after the new information becomes available. Prompt reporting of updates is required to allow the DHA Privacy Office to make timely final determinations on any subsequent notifications or reports. The Business Associate shall provide updates to the same parties as required for the initial Breach Reporting Form. The Business Associate is responsible for reporting all information needed by the DHA Privacy Office to enable timely and accurate determinations on reports to HHS as required by the HHS Breach Rule and reports to the Defense Privacy, Civil Liberties, and Transparency Division as required by DoD Privacy Program Issuances.

(b) Individual Notification Provisions

If the DHA Privacy Office determines that individual notification is required IAW 5 CFR §§ 164.400-414, the Business Associate shall provide written notification to individuals affected by the breach as soon as possible, but no later than ten working days after the breach is discovered and the identities of the individuals are ascertained. The ten-day period begins when the Business Associate determines the identities (including addresses) of the individuals whose records were affected.

The Business Associate’s proposed notification to be issued to the affected individuals shall

be submitted for approval to the DHA Privacy Office. Upon request, the Business Associate shall provide the DHA Privacy Office with the final text of the notification letter sent to the affected individuals. PII shall not be included with the text of the letter(s) provided. Copies of further correspondence with affected individuals need not be provided unless requested by the DHA Privacy Office. Pursuant to 45 CFR §§ 164.400-414 and section 13407 of the HITECH Act, the Business Associate's notification to the individuals, at a minimum, shall include the following:

- The individual(s) must be advised of what specific data was involved. It is insufficient to simply state that PII has been lost. Where names, Social Security Numbers (SSNs) or truncated SSNs, and Dates of Birth (DOBs) are involved, it is critical to advise the individual of the nature and extent of any potentially PHI data elements that have been breached. In all cases, individuals should be notified as to the nature and extent of any compromised PHI.
- The individual(s) must be informed of the facts and circumstances surrounding the breach. The description should be sufficiently detailed so that the individual clearly understands how the breach occurred.
- The individual(s) must be informed of any steps the individuals should take to protect themselves from potential harm resulting from the breach.
- The individual(s) must be informed of what protective actions the Business Associate is taking or the individual can take to mitigate against potential future harm. The notice must refer the individual to the current Federal Trade Commission (FTC) web site pages on identity theft and the FTC's Identity Theft Hotline, toll-free: 1-877-ID-THEFT (438-4338); Teletype (TTY): 1-866- 653-4261.
- The individual(s) must also be informed of any mitigation support services (e.g., one year of free credit monitoring, identification of fraud expense coverage for affected individuals, provision of credit freezes, etc.) that the Business Associate may offer affected individuals, the process to follow to obtain those services, and the period of time the services will be made available, and contact information (including a phone number, either direct or toll-free, e-mail address and postal address) for obtaining more information.

Business Associates shall ensure any envelope containing written notifications to affected individuals are clearly labeled to alert the recipient to the importance of its contents, e.g., "Data Breach Information Enclosed," and that the envelope is marked with the identity of the Business Associate and/or subcontractor organization that suffered the breach. The letter must also include contact information for a designated point of contact (POC), phone number, email address, and postal address.

If the Business Associate determines that it cannot readily identify, or will be unable to reach, some affected individuals within the ten-day period after discovering the breach, the Business Associate shall so indicate in the initial or updated Breach Report Form. Within the 10-day period, the Business Associate shall provide the approved notification to those individuals

who can be reached. Other individuals must be notified within ten days after their identities and addresses are ascertained. The Business Associate shall consult with the DHA Privacy Office, which will determine which media notice is most likely to reach the population not otherwise identified or reached. The Business Associate shall issue a generalized media notice(s) to that population in accordance with DHA Privacy Office approval.

The Business Associate shall, at no cost to the government, bear all costs associated with a breach of PII/PHI that the Business Associate has caused or is otherwise responsible for addressing.

VI. Termination

(a) Termination. Noncompliance by the Business Associate (or any of its staff, agents, or subcontractors) with any requirement addressed in this BAA may subject the Business Associate to termination under any applicable default or other termination provision of the Agreement.

(b) Effect of Termination.

(1) If the Agreement has records management requirements, the Business Associate shall handle such records in accordance with the records management requirements. If the Agreement does not have records management requirements, the records shall be handled in accordance with paragraphs (2) and (3) below, unless the Agreement has provisions for transfer of records and PII/PHI to a successor Business Associate, or if DHA gives directions for such transfer. In the case DHA or the Agreement provides for transfer of records, the Business Associate shall handle such records and information in accordance with such Agreement provisions or DHA direction.

(2) If the Agreement does not have records management requirements, except as provided in the following paragraph (3), upon termination of the Agreement, for any reason, the Business Associate shall return or destroy all PHI received from the Covered Entity, or created or received by the Business Associate on behalf of the Covered Entity that the Business Associate still maintains in any form. This provision shall apply to PHI that is in the possession of subcontractors or agents of the Business Associate. The Business Associate shall retain no copies of the PHI or its derivatives.

(3) If the Agreement does not have records management provisions and the Business Associate determines that returning or destroying the PHI is infeasible, the Business Associate shall provide to the Covered Entity notification of the conditions that make return or destruction infeasible. Upon mutual agreement of the Covered Entity and the Business Associate that return or destruction of PHI is infeasible, the Business Associate shall extend the protections of the Agreement to such PHI and limit further uses and disclosures of such PHI to those purposes that make the return or destruction infeasible, for so long as the Business Associate maintains such PHI.

VII. Execution

(a) Survival. The obligations of Business Associate under the “Effect of Termination” provision of this BAA shall survive the termination of the Agreement or any part thereof.

(b) Interpretation. Any ambiguity in the Agreement shall be resolved in favor of a meaning that permits the DoD Component and the Business Associate to comply with the HIPAA Rules and the DoD HIPAA Rules.

[Business Associate]

[DoD Covered Entity] Date

Date

APPENDIX C

DHA NON DISCLORE AGREEMENT

DRAFT

APPENDIX D

APPLICABLE PUBLICATIONS AND FORMS

DRAFT

APPENDIX E

HEALTH INSURANCE PORTABILITY AND ACCOUNTABILITY ACT (HIPAA)

DRAFT

APPENDIX F

CONTRACTOR FULL-TIME EQUIVALENT REPORTING ADDENDUM (CONTRACTOR MANOWER REPORTING APPLICATION (CMRA))

DRAFT

ATTACHMENT 1 – LIST OF MTFs

MTF Name
ACH Vilseck
Altus AFB (97th Medical Group)
Andersen JB (36th Medical Group)
Andrews AFB (79th Medical Group)
Aviano AB (31st Medical Group)
Barksdale AFB (2nd Medical Group)
Beale AFB (9th Medical Group)
Buckley AFB (460th Medical Group)
Camp Humphreys (Brian Allgood Army Community Hospital)
Camp Zama (BG CRAWFORD)
Cannon AFB (27th Medical Group)
Charleston JB (628th Medical Group)
Columbus AFB (14th Medical Group)
Davis Monthan AFB (355th Medical Group)
Dover AFB (436th Medical Group)
Dyess AFB (7th Medical Group)
Edwards AFB (412th Medical Group)
Eglin AFB (96th Medical Group)
Eielson AFB (354th Medical Group)
Ellsworth AFB (28th Medical Group)
Elmendorf AFB (673rd Medical group)
F.E. Warren AFB (90th Medical Group)
Fairchild AFB (92nd Medical Group)
Ft. Belvoir Community Hospital
Ft. Benning F.K.A Ft. Moore (Martin-Benning ACH)
Ft. Bliss (William Beaumont Army Medical Center)
Ft. Bragg F.K.A. Ft. Liberty (Womack Army Medical Center)
Ft. Campbell (Blanchfield Army Community Hospital)
Ft. Carson (Evans Army Community Hospital)
Ft. Drum (Guthrie Army Health Clinic)
Ft. Eustis (McDonald Army Health Clinic)
Ft. Gordon F.K.A Ft. Eisenhower (Eisenhower-Gordon AMC)
Ft. Hood F.K.A Ft. Cavazos (Darnall AMC)
Ft. Huachuca (Bliss Army Health Clinic)
Ft. Irwin (Weed Army Community Hospital)
Ft. Jackson (Moncrief Army Health Clinic)
Ft. Knox (Ireland Army Health Clinic)
Ft. Leavenworth (Munson Army Health Clinic)

Ft. Lee F.K.A Gregg-Adams (Kenner AHC)
Ft. Leonard Wood (Wood Army Community Hospital)
Ft. Lewis (Madigan Army Medical Center)
Ft. Meade (Kimbrough Ambulatory Care Center)
Ft. Polk F.K.A. Ft. Johnson (Bayne-Jones ACH)
Ft. Riley (Irwin Army Community Hospital)
Ft. Rucker F.K.A Ft. Novosel (Lyster AHC Clinic)
Ft. Sam Houston (Brooke Army Medical Center)
Ft. Shafter (Tripler Army Medical Center)
Ft. Sill (Reynolds Army Health Clinic)
Ft. Stewart (Winn Army Community Hospital)
Ft. Wainwright (Bassett Army Community Hospital)
Goodfellow AFB (17th Medical Group)
Grand Forks AFB (319th Medical Group)
Hanscom AFB (66th Medical Group)
Hill AFB (75th Medical Group)
Holloman AFB (49th Medical Group)
Hurlburt Field (1st Special Operations Medical Group)
Incirlik AB (39th Medical Group)
Joint Base Pearl Harbor-Hickam (15th Medical Group)
Kadena AB (18th Medical Group)
Keesler AFB (Medical Group)
Kirtland AFB (377th Medical Group)
Kunsan AB (8th Medical Group)
Lackland AFB (59th Medical Wing)
Landstuhl Regional Medical Center
Langley AFB (633rd Medical Group)
Laughlin AFB (47th Medical Group)
Little Rock AFB (19th Medical Group)
Los Angeles AFB (61st Medical Group)
Luke AFB (56th Medical Group)
MacDill AFB (6th Medical Group)
Malmstrom AFB (341st Medical Group)
Maxwell AFB (42nd Medical Group)
McConnell AFB (22nd Medical Group)
McGuire AFB (87th Medical Group)
Minot AFB (5th Medical Group)
Misawa AB (35th Medical Group)
Moody AFB (23rd Medical Group)
Mountain Home AFB (366th Medical Group)
Naval Hospital Naples

Naval Hospital Rota
Nellis AFB (99th Medical Group)
NH 29 Palms
NH Beaufort
NH Bremerton
NH Camp Pendelton
NH Guam
NH Guantanamo Bay
NH Jacksonville
NH Okinawa
NH Pensacola
NH Sigonella
NH Yokosuka
NHC Annapolis
NHC Charleston
NHC Cherry Point
NHC Corpus Christi
NHC Hawaii
NHC Lemoore
NHC New England
NHC Oak Harbor
NHC Patuxent River
NHC Quantico
NMC Camp Lejeune
NMC Portsmouth
NMC San Diego
Offutt AFB (55th Medical Group)
Osan AB (51st Medical Group)
Patrick AFB (45th Medical Group)
Peterson AFB (21st Medical Group)
RAF Lakenheath (48th Medical Group)
Ramstein AB (86th Medical Group)
Redstone Arsenal (Fox Army Health Clinic)
Robins AFB (78th Medical Group)
Scott AFB (375th Medical Group)
Seymour Johnson AFB (4th Medical Group)
Shaw AFB (20th Medical Group)
Sheppard AFB (82nd Medical Group)
Spangdahlem AB (52nd Medical Group)
Tinker AFB (72th Medical Group)
Travis AFB (60th Medical Group)

Tyndall AFB (325th Medical Group)
USAF Academy (10th Medical Group)
Vance AFB (71st Medical Group)
Vandenberg AFB (30th Medical Group)
Walter Reed National Military Medical Center
West Point (Keller Army Community Hospital)
Whiteman AFB (509th Medical Group)
Wright Patterson AFB (88th Medical Group)
Yokota AB (374th Medical Group)